

COVERAGE & CONTINUITY

Reciprocal Coverage Agreement

A signable two-clinician agreement for covering each other's patients during planned and unplanned absences, with an activation form and debrief note.

AUDIENCE

Psychologists · Psychiatrists

FORMAT

Agreement + 2 exhibits

VERSION

1.0 · Sept 2026

TIME TO ADAPT

20–30 minutes

WHAT IT IS

A plain-language agreement in which two licensed colleagues cover each other's patients for vacations, illness and emergencies, plus a one-page coverage summary and a debrief note.

USE IT WHEN

You want a standing arrangement with a trusted peer, or you are joining a group and want the informal "buddy system" put in writing.

CUSTOMIZE FIRST

Licensure in each patient's location, your malpractice policy terms, payer billing rules, and how patients are told about coverage in your informed consent.

Why put coverage in writing

Most clinicians have a friend who "covers" for them. Few have it on paper. When something goes wrong (a patient in crisis while you are unreachable, a billing question, a records request), an informal understanding leaves both clinicians and the patient exposed. The APA Ethics Code expects psychologists to make reasonable efforts to plan for interruptions of service caused by illness, death, unavailability, relocation or retirement (Standards 3.12 and 10.09, with 6.02(c) on records). A written agreement is the simplest evidence that you did.

- It names who covers whom, for what, and for how long.
- It sets what information is shared, how, and when.
- It settles billing, liability and insurance questions before they become disputes.
- It turns coverage into a five-minute activation routine instead of a scramble.

CHOOSE THE COLLEAGUE FIRST

Coverage works best between clinicians with overlapping competence, similar risk tolerance and compatible communication styles. Meet once before signing. Ask each other: How do you handle a patient who calls in crisis? What is your realistic response time? What kind of patient would you decline to cover?

Part A: The agreement

Written for two clinicians (the "Colleagues"). For three or more, use it pair by pair. Complete the parties block, choose options in section 10, then sign.

Parties

Colleague A: full name and credential

Practice name

License number(s) and state(s)

NPI

Phone for urgent coverage contact

Secure email or messaging address

Professional liability carrier and limits

PSYPACT / interstate status (if applicable)

Colleague B: full name and credential

Practice name

License number(s) and state(s)

NPI

Phone for urgent coverage contact

Secure email or messaging address

Professional liability carrier and limits

PSYPACT / interstate status (if applicable)

1. Purpose

The Colleagues agree to cover each other's patients during the other's absence, planned or unplanned, on the terms below. "Coverage" means being reachable for urgent clinical needs and crisis contact, and taking the reasonable steps needed to protect a patient until the treating Colleague returns or care is transferred.

2. Types of coverage

Type	What the covering Colleague does	Typical length
Short planned absence (vacation, conference)	Urgent contact only. No routine sessions unless agreed for a specific patient.	Up to 14 days
Unplanned absence (illness, emergency)	Urgent contact and crisis assessment as needed. Helps notify patients and triage the schedule.	Days to weeks
Extended planned leave (parental, medical, sabbatical)	Follow the Extended Leave Coverage Plan. May include temporary transfer of routine sessions.	Over 14 days
Prescribing coverage (psychiatrists)	Urgent medication questions and bridging prescriptions within the covering prescriber's scope, license and DEA registration.	As needed

3. What coverage does not include

Unless the Colleagues agree in writing for a specific case: (a) taking over ongoing routine psychotherapy; (b) forensic, custody or disability evaluations; (c) prescribing controlled substances where the covering Colleague is not licensed and DEA-registered for the patient's location; (d) patients outside the covering Colleague's area of competence; (e) patients located where the covering Colleague is not licensed or otherwise legally authorized to practice.

4. Licensure and patient location

The covering Colleague serves only patients who are physically located, at the time of service, in a jurisdiction where the covering Colleague is licensed or otherwise legally authorized. For psychologists using PSYPACT, the Authority to Practice Interjurisdictional Telepsychology (APIT) generally requires the psychologist to be physically in their declared PSYPACT home state while providing services. At activation, the treating Colleague identifies which patients are out of state.

5. Patient notice and consent

Each Colleague names the other as a covering colleague in their informed consent. Suggested wording: "While I am away I may arrange for a colleague, [Name], to be available for urgent needs and may share the information needed for your safety and care." HIPAA generally permits one provider to disclose information to another for treatment purposes without a signed authorization, but state law may be stricter and patients trust the process more when they are told in advance. Psychotherapy notes, as HIPAA defines them, are never shared under this Agreement.

6. Information shared

For each patient the covering Colleague may need to know about, the treating Colleague provides a one-page Coverage Summary (Exhibit A) limited to the minimum necessary. It travels by secure channel only: encrypted messaging, an EHR share, or a HIPAA-compliant portal. No patient information goes by ordinary text or unencrypted email.

7. Availability and response

During coverage the covering Colleague will return urgent calls within [2] hours between [hours] and will give patients an after-hours route or direct them to 911 and to 988 (call, text or chat). Neither Colleague is obliged to be available around the clock. Each will tell patients honestly what to expect.

8. Documentation and return

The covering Colleague documents every contact in the patient's record or, without record access, in a dated contact note sent to the treating Colleague within [1] business day. On return, the Colleagues hold a 10 to 15 minute handoff covering contacts, changes in risk, medication changes and follow-up needed, recorded in the Debrief Note (Exhibit B).

9. Conflicts

If a Colleague has a personal or professional conflict with a patient (for example, a dual relationship), they will say so and decline to cover that patient.

10. Billing and fees (choose one)

- ☐ No charge between Colleagues. Urgent coverage contact is not billed.
- ☐ The covering Colleague bills the patient under their own name and NPI, at their usual rate, for services they personally deliver.
- ☐ Other arrangement: [describe]

BILLING TRAPS

Never bill under the absent Colleague's NPI for services you personally deliver unless the payer's contract or manual expressly permits a reciprocal or locum arrangement. Check each payer. Neither Colleague pays or receives anything for referring a patient to the other (see APA Ethics Code Standard 6.07 on fees and referrals).

11. Professional liability

Each Colleague keeps professional liability insurance with limits of at least [\$ per claim / \$ aggregate], will tell their carrier about this arrangement if the policy requires it, and will give the other a current certificate by [date]. This Agreement creates no partnership, employment or agency relationship. Each Colleague remains solely responsible for their own professional acts.

12. Confidentiality

Each Colleague is bound by HIPAA and applicable state law, will use information received under this Agreement only for coverage, and will securely return or delete it when coverage ends, except what belongs in the clinical record.

13. Term and ending

This Agreement runs for [12] months and renews automatically unless either Colleague gives [30] days' written notice. Either may pause it for a specific absence by telling the other. Coverage already begun continues until patients are safely transitioned.

14. Governing law and disagreements

[State] law governs. The Colleagues will first talk, then involve a neutral colleague they both trust, before any formal process. Each has had the chance to take advice from their own attorney and malpractice carrier.

Colleague A signature

Date

Colleague B signature

Date

Exhibit A: Coverage Summary (one per patient)

Complete for each patient the covering Colleague may need to support. Share by secure channel. Update if anything changes before you leave.

Patient name or ID	Phone and preferred contact method
Emergency contact and relationship	Physical location / state for telehealth
Working diagnosis / presenting concern	
Treatment approach in one line	
Current risk status (low / moderate / high) and basis	
Safety plan on file? Firearms / lethal means notes	
Medications and prescriber	Substance use flags
What helps this patient in a crisis call	
Cautions: what to avoid	
Patient told about coverage on (date)	Consent documented in record

Exhibit B: Coverage activation and debrief

Treating Colleague		Covering Colleague
Coverage start	Coverage end	Patients flagged (initials or IDs)

Debrief on return

Number and type of contacts

Any crisis events or changes in risk

Medication changes or prescriptions issued

Actions needed from the treating Colleague

What to change in this Agreement

Debrief completed (date)	Initials of both Colleagues
--------------------------	-----------------------------

Practice notes

- Trial it on one short absence first, then debrief and adjust.
- Review it every year alongside your informed consent and your professional will.
- Agree a backup for when your Colleague is also away.
- Tell patients early. Patients who already know the covering Colleague's name are far more likely to call them.

IN PSYALLIANCE

Find covering colleagues through Network > Partners and filter by state, specialism and PSYPACT status. For longer absences, the Planner drafts coverage outreach for you.

Sources and further reading

- APA Ethics Code, Standards 3.12, 6.02, 6.07 and 10.09 ([apa.org/ethics/code](https://www.apa.org/ethics/code)).
- HHS, HIPAA Privacy Rule: uses and disclosures for treatment ([hhs.gov/hipaa](https://www.hhs.gov/hipaa)).
- PSYPACT Commission, telepsychology authorization requirements (psypact.gov).

ABOUT THIS DOCUMENT

Prepared by PsyAlliance from publicly available professional and regulatory sources, reviewed against them as of September 2026. It is an educational template and practice aid. It is not legal, ethical, clinical or billing advice, and it does not create a professional relationship. Laws, licensing-board rules and payer requirements vary by state and change over time. Verify against your licensing board, professional association, payer contracts and, where the stakes warrant it, a qualified attorney before relying on it.

Provided to PsyAlliance members for adaptation to their own practice. Please do not resell or republish. Text in [highlighted brackets] is a placeholder for you to complete.